

Eczema (atopic dermatitis)

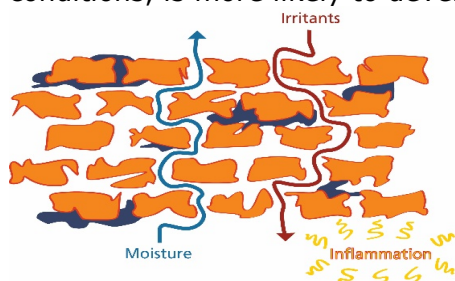
What is eczema?

Eczema, or atopic dermatitis, is a common skin problem in children. About 1 in 3 children under 5 years of age have eczema. It may be worse for the first 2 years of life. The exact cause is not known, but often the child and other family members will have allergic conditions, e.g. hay fever or asthma.

Children with eczema have dry, sensitive skin, which becomes itchy. This leads to scratching which means the skin becomes sore and cracked. If the skin then becomes infected the eczema is harder to treat. At times, your child's skin will be good and other times the eczema will flare up (inflammation). This is part of the condition. Flare-ups are more common in winter and when children have mild infection, e.g. a 'cold' or babies are teething.

What is the cause?

It is not well understood why some people develop eczema and others don't. A person with a history of asthma, food allergy as a baby or hay fever, or a family history of these conditions, is more likely to develop eczema.



Dry, itchy skin with eczema has less oils and fats in the skin. A damaged barrier lets moisture escape; irritants get in

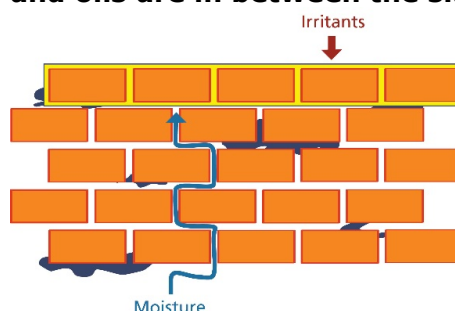
In people with eczema the skin barrier works less well than in people without eczema. The skin dries out easily. Irritants get into the gaps in the barrier, leading to redness, irritation and itch. Using moisturiser often and avoiding irritants are the most important ways to control eczema.

How long does it last?

Eczema is a chronic condition (lasts a long time) and may persist to adolescence or adulthood.

We can help you learn how to get the skin back to being soft and having a barrier there is no "cure" a lot can be done to control it.

Below, shows how the skin works in people who don't have eczema. No gaps in the skin barrier and one's own fats and oils are in between the skin cells.



Is it due to foods?

In some infants with eczema, some foods cause eczema to flare up. If you suspect a food is causing your child's flare-ups, talk to your doctor about referral to an allergy specialist who can find out whether there is a food allergy. Try not to restrict your child's foods before you have the allergist's advice and/or an accredited dietitian with special knowledge in allergies. Food restriction without medical supervision may cause your child to miss out on important nutrients. This may lead to food intolerance. It is wise to avoid moisturisers containing foods, plants or flowers (e.g. sunflower, calendula)

If you seek advice from natural therapists, it is important to check their training and credentials, especially if they suggest food restrictions for your child and/or perform "allergy testing".

How to reduce or prevent eczema flares. Bathing

Children with eczema always have dry skin, but after a bath, the skin is hydrated and feels good. Bathe your child in a lukewarm (not hot) bath for 10 minutes daily then pat (not rub) the skin half-dry with a soft towel.

Use non-soap based wash. Do not use soap; even 'moisturising soap' dries the skin. Eczema skin, including hands, is very sensitive to soaps and bubble bath. Teenagers should still use a gentle cleanser to wash under the arms, genitals, feet.

Hair

Most **non-soap-based wash** can be used as **shampoo**. Commercial shampoos bubble up and dry out the head and facial skin (and hands, as you use them to wash the hair). If you believe shampoo is important, use one recommended for people with eczema (eg. QV or Dermaveen).

Moisturising

After bathing or showering and at other times of the day, apply a **thick** moisturising cream or ointment

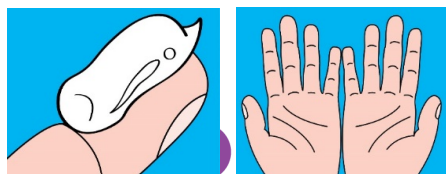
all over your child's skin. Putting moisture back into the skin and keeping it there is VITAL to control eczema. Wash your hands before applying. Use a spatula or spoon to remove cream from the tub. Do not use lotion as these are runny and often sting the skin.

How much moisturiser do I use?

Minimum use - twice daily
Volume to use per week

500 gms for adults/teenager
250 gms for child/baby

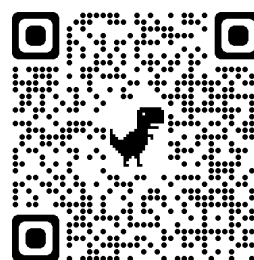
Very-hydrating creams or ointments are QV balm, Epaderm, QV Intensive with Ceramides (non-sting only), Dermeze, Dermaveen or CeraVe. In summer, it may help to keep these in the fridge; applying cool cream can soothe skin.



Medicated baths

If eczema becomes infected the doctor may suggest swimming in a pool for your child. If you do not have a pool, you can make up the pool water solution at home. Very dilute bleach works like pool water and may help settle the skin infection which then may lessen the itch. Use the recipe found at the QR Code below.

If you do not have a bath tell the doctor or nurse. They will explain what to do in the shower.



Or Google "RCH bleach baths" for instructions on diluting the bleach

Treating the flare-ups:

Cortico-steroid (cortisone) creams and Ointments

Sometimes a doctor may prescribe a cortico-steroid ('steroid' or cortisone) cream or ointment to help settle the inflammation (redness) and itch of eczema when a flare up occurs. Although your child's body produces its own cortisone, when a flare happens, the body needs extra to help turn down the flare. The help is the Cortico-steroid ointments which are safe when used as directed by your doctor.

You should put the cortisone (steroid) cream/ointment on **BEFORE** the moisturiser. Some doctors suggest a time gap between putting on the steroid and moisturiser. There is different evidence on this. It may be sometimes difficult to get your child to wait. What is most important is that the prescribed creams are applied.



How much steroid cream to put on?

When you put on the cream, put enough on to leave a 'shine' on your child's skin. Squeeze cream onto your fingertip (wear gloves if possible) like shown in the picture.

1 Finger-tip of steroid cream covers the area of two adult hands

Children may need 2 types of steroid creams—one is weaker than the other. Make sure your doctor and/or nurse **explains** to you which one to use on what body areas. Also, ask the doctor or nurse **how long you need to use the steroid** for.

Today's recommendations:

Facial / mild steroid cream

Your child's steroid cream is

Apply _____

Apply this cream 2 times a day when the skin is rough, red and itchy, once a day if only mildly red or rough.

This cream is mild enough for the face but can also be used on other areas when the skin is not severely affected.

Stronger steroid cream (body only)

This is for severe flare-ups only and is **too strong** to use on the face or genital area. Make sure the nurse or doctor helps you understand how long to use the creams.

Your child's **stronger** cream is

Apply this cream twice a day when the skin is both rough and red, once a day if only mildly red or rough. **Never apply this more powerful steroid cream to the face or genital area without a doctor's advice.**

Once the flare is improving you may need to gradually reduce how much and how often this cream is used (so-called "weaning").



Change back to the weaker steroid cream when the severe flare-up is over, or stop as your doctor/nurses advised.

Preventive non-steroid cream

(e.g. pimecrolimus) This cream is usually only prescribed by a dermatologist (skin specialist) or allergy specialist.

Itching

At the first sign of any itching, apply moisturiser. You may need to use an ointment. It must be thick enough that it does not fall out of the tub when you turn the tub upside down with the lid off. Use the steroid cream if redness or raised and/or itchy patches develop. Keep your child's fingernails cut short or consider having them wear mittens or special splints to bed if the itching is severe. Also, wash your child's hands with water and a non-soap based wash often to avoid infecting the eczema.

Antihistamine (anti-itch) medicine

An antihistamine may be helpful to reduce night time itch. Antihistamines suitable for children over 6 months of age include Cetirizine (Zyrtec), Loratidine (Claratyne) or Fexofenadine (Telfast). Your pharmacist can assist with advice about how much to give your child. Diphenhydramine (Benadryl) may make your child sleepy so it may be a good choice at night time, but only for children over 5 years of age.

When should I see a doctor?

Urgently if:

- Your child has a fever.
- The rash flares up or blisters, after your child has been in contact with someone who has cold sores.

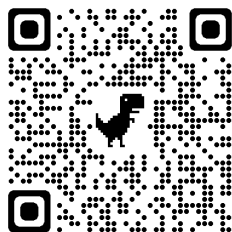
Within 24 hours if:

- The rash becomes raw and open
- The rash looks infected (pus, yellow scabs or pain are present)
- The rash hasn't greatly improved in 7 days of treatment



Eczema Plan

Have your doctor fill in the **Step wise eczema plan** found at this QR code or at www.ascia.org.au



Other tips

- Use loose, mostly cotton clothes where possible. Wool fibre, scratchy, rough, synthetics make eczema worse.
- When it is cold, use layers of light clothes, not heavy clothes.
- Avoid overheating any room, using electric blankets or woollen under-blankets. Try to use soft cotton or satin-like sheets.
- Avoid triggers that cause eczema to flare up, e.g. excessive heat, sweating, cold, dry air (use a humidifier), chlorine, chemicals, soaps.
- Avoid laundry detergents with enzymes. Look for products labelled 'sensitive', 'gentle' or 'Eco-friendly'. Rinsing twice removes detergent
- Minimise your child's grass contact in pollen season (September-December). Moisturise skin before playing on grass
- Keep your child away from anyone with cold sores. The **herpes virus can cause a serious skin infection** in children with eczema.
- Cover the skin with moisturiser before swimming.
- Cover skin with greasy moisturiser before going in sea water as it may sting
- Try to breast-feed all high-risk infants at least until they are 6 months old



For more information

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<http://www.wch.sa.gov.au> September 2021



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This document has been reviewed
and endorsed by WCHN consumers

Wet dressings/wraps

These are used when your child's itching gets bad, or if the skin is very cracked and sore. Wet wraps help the steroid cream and moisturiser work better and protect the skin from scratching and infection.

For wet dressings/wraps: use warm water to dampen some soft, stretchy fabric such as Chux wipes, T-shirts, leggings, pyjamas (PJs) or Tubifast (a special stretch material like a stocking).

Place the damp fabric over the eczema **after the creams have been applied.**

To keep the wet dressing in place you can use dry Tubifast to fit the arms and legs, crepe bandages, Gro-suit, a T-shirt, or leggings, depending on where the eczema is and what suits your child.

My child hates wet wraps

If your child hates the damp feel, put on the bandages/tubifast dry. Then make the dry dressing damp by spraying water on using a clean spray bottle containing warm, clean water. Spray enough water on to fabric damp, not dripping with water.

If the eczema is on your child's face or head, you can hold a wet dressing to the area for 5-10 minutes. Wet dressings to the face or head should not be bandaged.

When the dressings dry out, they will make the skin hot and itchy. Either remove them or re-moisten them with the spray bottle warm water. The doctor or nurse will explain wet wraps. Scan this QR code for the short video on wet wraps. Clips also show you how to apply moisturiser, cortisone cream, and prepare bleach baths.

